



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : عزه مرسى ابراهيم بيصار
المحافظة : المنوفية الحي / المدينة : شبين الكوم
رقم البطاقة : 27808011700623 رقم التليفون : 01002015957
العنوان : ه
العمر (تاريخ الميلاد) : 01-08-1978 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي (.....)
المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **عزہ مرسى ابراهيم بىصار**
National ID: **27808011700623**



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Bone scintigraphy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

عزة
مرسى ابراهيم ببصار
٩ ش ابراهيم من طلعت حرب
شبين الكوم - المنوفية

١٩٧٨/٠٨ ٢٧٨٠٨٠١١٧٠٠٦٢٣

LA8581102

عامل
DMS
مسئول زري عدا خفام

٢٠٢٥/٠٣ ١٧٨٠٨٠١١٧٠٠٦٢٣
دبلوم معهد فني كوبري
أنش
حسن كمال القيسي بغداد
البطاقة مازولة حتى ٢٠٢٢/٠٣/١٥

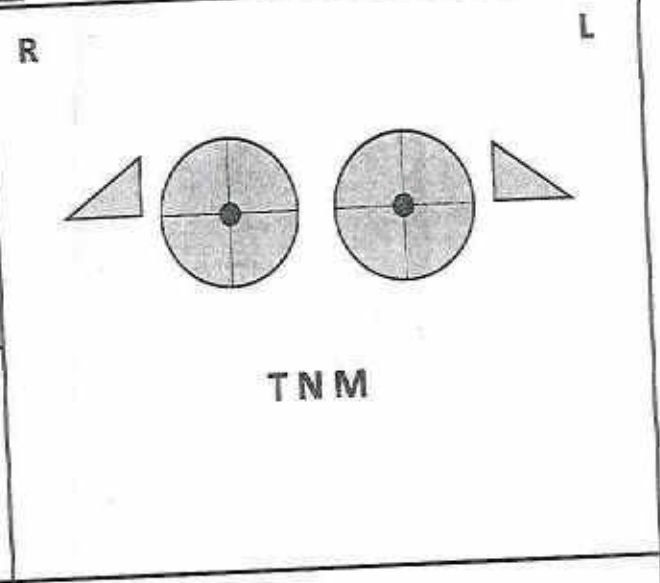
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MDT FORM 31 / 12 / 2025 G

Name	عزوة حرس إبراهيم	MRN	1882/2025
Age	45 yrs	ID	
Menopausal status		Phone	01002015957
NRS SCORE		Marital status	
BMI		Family history	ve
CO morbidity	-ve	REASON OF MDT	PSI

Clinical Data

BX:			
ER:	PR:	HER 2:	Ki67:
818	818	-ve	5%
Sonomammography:			
Mets Workup:			



Recommendations: 45 yrs old

Denovo metastatic breast cancer since 2022
 Received Femara (Zoladex) (2022 - 5/2025) 1 DP of bone.
 Started Faslodex / Ibrance (Received only 2 cycles) (between 7/2023 & 12/2023)
 New Follow up showed - 35% new foci at L4 rib.

MDT OUTCOME & PLAN

After reviewing pri the pt was irregl on CDK4/6 " 2 cycles
 pt received only 2 cycles CDK4/6
 Recommended to keep on it
 for 3 more cycles (total 5 cycles)
 31/12/2025

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name: عزہ مرسي ابراهيم بيسار		
MR. : 1249		
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was diagnosed with metastatic breast cancer to bone in 2022 and initiated first-line therapy with femara and Zoladex. The treatment plan was subsequently modified, and by July 2025 the multidisciplinary team recommended a regimen of Faslodex plus Ibrance. On 21 January 2026 the patient received cycle 4 of Faslodex and cycle 3 of Ibrance 125 mg. In May 2026 a CT of the chest, abdomen, and pelvis was performed, which was deemed stable disease for Ibrance/Faslodex. A bone scan was also ordered, with the next follow-up scheduled for 22 June 2026. In June 2026 the CT chest/abdomen/pelvis continued to show stable disease, and the bone scan performed in June was pending radiology department approval. In July 2026 the patient presented for administration of Faslodex and Ibrance 125 mg; laboratory results were acceptable, and the next clinic visit was scheduled for 24 August 2026. In August 2026 the patient's total leukocyte count was $2 \times 10^9/L$ and absolute neutrophil count $0.7 \times 10^9/L$, prompting a postponement of treatment by one week. Faslodex 500 mg intramuscular was planned as the only therapy, with the next visit set for 30 September 2026.		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	
	نهي سعيد علي محمد	الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذة / غادة موسى إبراهيم يونس
Patient No : 28708011700823
Path No. : BR-913/25
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13004038
Request Date : 02/07/2025
Verification Date : 09/07/2025
Financial Category : اورام ذاتي السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (11201 A), together with six referred slides including 2 H&E stained slides and 4 immunostained slides for (ER, PR, HER2 & Ki67).

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- BSA.

Microscopic Examination:

- Histological type: invasive lobular carcinoma.

- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).

- DCIS: Not detected.

- LCIS: Minimal component.

- Angiolymphoma: Not detected.

- Periductal myxoma: Not detected.

- IHC:

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).

- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).

- Ki67: Not detected.

(Average Ki67: 1.7; Her2/CE17 ratio: (1.8)).

- Ki67: 5%.

Validated by:

Dr. Dina Mohamed

Printing Date: 13/07/2025 10:31:53

Page 1 of 3

Printing by: marwa mohamed youss

شركة رفيع (4) - الطور الاول - مبانى رقم 105 - كورنيش البحر - جسر الكوبري - القاهرة - مصر
Korumbou (4) - 1st Floor - Bldg No. 105 - Bld. Corniche - Nile El Qadima - Cairo, Egypt

Patient Name	الأستاذة / عزه مومني إبراهيم يوسف	Request No.	: 13004038
Patient No	: 8708011700623	Request Date	: 02/07/2025
Path No.	: SR-913/25	Verification Date	: 09/07/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hagezy

Pathology Report

Diagnosis

Left Breast mass: Referred block & slides (Tru-cut biopsy):
 - Invasive lobular carcinoma, Grade II.
 IPI:
 - ER: Positive (score 8/8).
 - PR: Positive (score 8/8).
 - Ki67: Equivocal (score 2+).
 - Her2: Not tested, Group V.
 Average Her2 copy No.: (2.85), Her2/CEP17 ratio: (1.8).
 (65/16, 1%).

* Grading: ER, PR
 ASCO-CAP Her2 Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1-10%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 55% and 5, between 56% and 100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, moderate; and 3, strong). Total score from 0 to 8.
 * Staining: Her2

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+ as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+ based on circumferential membrane staining that is incomplete and/or intense and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+ based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
 (See grade).

Verified by:
 Dr. Mohamed

Printing By: Amrwa Mohamed Youss

10/07/25
10:45

Patient Name : الأستاذة / غادة مرسى إبراهيم تيسعد
Patient No. : 00708011700623
Path No. : BR-913/25
Sex : Female
Age : 35 Y
Referring Doc :

Request No. : 13004038
Request Date : 02/07/2025
Verification Date : 09/07/2025
Financial Category : أورام دار السلام - نقية النوية
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4 (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 5 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 5. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4

Signature
Dr. Hossam Elwan (Shonam)
Dr. Hossam Elwan

Dr. Osama Mohamed

Printing Date : 09/07/25 10:31:53

Page 3 of 3

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Report No. (4) - First Floor-Building No. 987- Nile Corniche - Nisr El Qadima - Cairo, Egypt

Code:11201

Sex: Female

التاريخ : ٢١ - ٤ - ٢٠٢٢ م

Clinical Data: Left breast mass.

اسم المريض: عزة مرسى ابراهيم

السن: ٤٠ سنة

الطبيب المعالج: اد/ أحمد الكيلاني

Pathology report

Nature of the Specimen:

Tru cut biopsy.

Gross Appearance:

Specimen contained multiple grayish cores: 9-15 mm long, totally submitted.

Microscopic Appearance:

Sections reveal a breast tissue infiltrated by invasive carcinoma composed of cells arranged in linear single-cell (Indian filing) pattern in a sclerotic stroma. Neoplastic cells are small, round with lack of cohesion, slight pleomorphism, scanty cytoplasm, evenly disbursed chromatin and occasional nucleoli. There is no associated significant lobular neoplasia or intraductal component (DCIS).

Diagnosis:

LEFT BREAST MASS; TRU CUT BIOPSY:

- *Invasive mammary carcinoma, grade II.*

الختم
تحليل الأنسجة والأورام
أ.د. أسامة مصطفى

Code:11201

Sex: Female

التاريخ: ٢٦ - ٤ - ٢٠٢٢ م

Clinical Data: Left breast cancer.

اسم المريض: عزة مرسى ابراهيم
السن: ٤٠ سنة
الطبيب المعالج: اد/ أحمد الكيلاني

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for c-erb-2 (HER2/neu) using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

For the determination of HER2 protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumor are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells) and 1+ (faint/barely perceptible incomplete membrane staining in $> 10\%$ of tumor cells). The test is interpreted as weakly positive (border line or equivocal) for score 2+ (circumferential membrane staining that is incomplete and/or weak/moderate within $> 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $\leq 10\%$ of the invasive tumor cells). A strong positive result is interpreted when the score is 3+ (strong circumferential membrane staining in $> 10\%$ of invasive tumor cells).

Diagnosis:

IMMUNOSTAINING FOR C-ERB-2 (HER2/NEU):

- HER2/neu status: Negative (score 1)

Code: 11201

Sex: Female

التاريخ: ٢٦ - ٤ - ٢٠٢٢ م

Clinical Data: Left breast cancer.

اسم المريض: عزة مرسى ابراهيم

السن: ٤٠ سنة

الطبيب المعالج: اد/ أحمد الكيلاني

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for ER, PR and Ki-67 using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

ER: More than 80 % of tumor cells show strong nuclear staining (*Allred scoring system: 8/8*).

PR: More than 80 % of tumor cells show strong nuclear staining (*Allred scoring system: 8/8*).

Ki-67: 5 % of the tumor cells (*at the hot spots*) show strong nuclear staining.

Diagnosis:

ER, PR IMMUNOHISTOCHEMISTRY:

- **ER Status:** Strongly positive (*Allred scoring system: 8/8*)

- **PR Status:** Strongly positive (*Allred scoring system: 8/8*).

KI-67 IMMUNOHISTOCHEMISTRY:

- **Ki-67 labeling index:** 5 %.

٢٦ / ٤ / ٢٠٢٢
أحمد الكيلاني

Patient Name: Azza Morsi Ibrahim Bissar	Date: 18/11/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age: 47 Year(s)
Medical ID: 1882/2025	Pat ID: 133014

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF
THE CHEST, ABDOMEN AND PELVIS REVEALED:
COMPARED TO LAST STUDY DATED 8-5-2025 CURRENT
STUDY REVEALS**

I- CHEST:

- Stationary course as regard the last study dated 8-5-2025
- No sizable pulmonary nodules , masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

Compared to last study dated 8-5-2025 , current study reveals;

- Enlarged liver showing diffuse low parenchymal attenuation. It shows a newly developed tiny left hepatic lobe segment II enhancing focus measuring 4.8 mm in diameter showing Porto venous phase enhancement and delayed wash out.
- No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- As regard the last study **increased in size** is noted at the right adnexal cyst seen measuring **6.4 cm** compared to **5cm** in last study in diameter and **newly** developed right side adnexal cyst **2.8 cm** on the left side.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.

Name : Azza Mursi Ibrahim

Date : 10-11-2025 11:25

Branch : Dokki

Medical No. : 245889

Ref. Physician Prof : Gostaf Roseh Hospital

Ans. Company : تعافد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 47-year-old lady, with history of metastatic breast cancer and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Considering the prior scan dated 31/05/2025; the current study still showing active osseous lesions at multiple dorso-lumbar vertebrae and bilateral iliac bones.
- Newly-developed active foci at few left ribs. *على اليمين*
- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Multiple metastatic osseous lesions with progressive disease course evidenced by newly-developed deposits as described.

Best regards,

Dr. Alaa Nagi, MD
Nuclear Medicine

Name : Azza Mursi Ibrahim Beisar Date : 20-06-2026 07:24
Branch : Maadi Medical No. : 397688
Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : تعافى مششفيات

Whole-Body Bone Scintigraphy Report

Indication: Follow-up of known case of metastatic breast cancer in a 47-year-old female patient; evaluation of therapeutic response.

Technical Procedure: Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings

Axial Skeleton

- Persistent focal tracer hyperfixation throughout multiple dorsolumbar vertebrae (D11-L3). It demonstrates significant interval reduction in intensity and extent compared to the study dated 10/11/2025, representing scintigraphic regression.
- Faint, residual uptake remains at the prior left rib foci; no newly developed areas of focal osteoblastic activity are seen bilaterally.

Appendicular Skeleton & Pelvis

- Near-complete resolution and profound down-regulation of the intense tracer hyperfixation previously observed in the bilateral iliac crests and sacroiliac joints.
- Physiological, symmetric tracer distribution throughout the extremities; mild, diffuse periarticular uptake consistent with stable degenerative arthropathy.

Soft Tissue & Background

- Normal symmetric renal excretion and soft tissue background clearance.

Impression

In comparison with the prior studies dated 31/05/2025 and 10/11/2025:

- **Significant Regressive Disease Course:** Marked regression of the widespread osteoblastic skeletal metastases involving the dorsolumbar spine, bilateral iliac crests, and sacroiliac joints, evidenced by a profound reduction in tracer hyperfixation.
- **No New Metastatic Disease:** No scintigraphic evidence of newly developed focal osteoblastic lesions.
- **Overall Response:** The scintigraphic profile indicates an excellent partial therapeutic response to the ongoing systemic regimen.

Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name: Azza Morsi Ibrahim Beisar
Date: 2026-05-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

1249

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. Compared to the previous study dated 18-11-2025,
the current study showed:

I- CHEST:

- No appreciable time interval changes regarding the size and number of the previously seen few right upper and middle lobe pulmonary nodules largest still measuring 7 mm at the right middle lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing diffuse low parenchymal attenuation. Stationary course regarding the previously seen tiny left hepatic lobe segment II enhancing focus measuring 4.8 mm in diameter. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- Rather stationary course regarding the size of the bilateral adenexal cysts still seen measuring 6.4 cm in diameter on the right side and 2.8 cm on the left side.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric mesenteric and para-aortic lymph nodes.
- No ascites.
- No appreciable time interval changes regarding the multiple previously seen axial and appendicular skeletal lytic and sclerotic bone lesions.

Opinion:

❖ Stationary course since last study.

Dr.sherif Wagdy

DOCTOR'S ORDERS

Diagnosis: Unit: 45 y. Diet Type: *Gymnogeomys*

Date	Time Of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
	ER 8/8	2022	MBC	From start			
	PR 8/8		on Femora + Zoladex				
	hept -ve	7/2025 mdt:	Fasoladex				
			IbranCe 12S				
			CyFasoladex 500 TM				
			C3IbranCe 12S				
			calcium				
			bone care				
			Curem (iv)				
			C-CT/11/151				

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.

* Do NOT use abbreviations.

• This form is NOT intended for use for medications orders.

پیشہ ورانہ

- * All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
- * Do NOT use abbreviations.
- * This form is NOT intended for use for medications orders.

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٢٠٢٥



عزة
مرسى ابراهيم بيسار
٩ ش أبوالمعاطي من طلعت حرب
شبين الكوم - المتوفية



٢٧٨٠٨٠١١٧٠٠٦٢٣

LA8581102

تقرطان شدي
١٧١٢
د

٤٥
بالتة

شدي

١٨٢٠١٥٩٥٧



٢٠٢٥/٠٣ ٢٧٨٠٨٠١١٧٠٠٦٢٣
ديكوم معهد قلى تجارى

أنش
مستمة
متروجة
حسن كمال الغباشى بغداد
البطاقة سارية حتى ٢٠٢٢/٠٣/١٠



١٢



ERM036745486

الاسم: عزّة مرسى إبراهيم

التاريخ: ١٦/٩/٢٠٢٠



A case of Breast cancer was
on Zoladex, Femara
she developed Multiple Bns M
I recommend.

Lasloca + Ibrahim

+ Method ionic

+ Trimmer 100
12x2x1

Thank

~~2/15/21~~

9-90

الكشف بميعاد مسبق و الإستشارة مرة واحدة خلال اسبوعين

المنوفية - شبين الكوم - آخر مجمع المواقف - امام هايير السنطاوى

0482060367 - 01008680878 - 01080040338

Azza morsy Ebrahim DOB: 31-May-1985 ID: Bone Scan SEX: F
STUDY DATE: 31-May-2025

Patient Name: Azza morsy Ebrahim
Study Name:

Patient ID: Bone Scan
Study Date: 31-May-2025

DOB: 31-May-1985

Wholebody 31-May-2025

Wholebody 31-May-2025



التشخيص: نوع التغذية:

توقيع الطبيب	الأوامر	الوقت	التاريخ
	23/2025 PS=0 45 yrs old the son (her brother) has cancer MBC to bone from stat n Zoladex - Fencar Svc 2022 5/2025 → IT bone For faslodex - Thence 125mg 22/7/2025 22/7/2025 At will rec Zoladex 2 Zonta outbid 17-8-2025 Pt received 1st faslodex 4/8/2025 Time for 2nd faslodex loading		

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

- Ibuprofen 125 ale CRS
- cat
- one sh.

.....: **التشخيص:**: **نوع التغذية:**

جميع الأوامر الطبية لابد أن تكون واضحة وموقعة من الطبيب

MDT FORM

21/7/2025 G

Name	عزوة محمد ابراهيم	MRN	1888100
Age	45 yr	ID	00000000000000000000
Menopausal status	1	Phone	01000000000
NRS SCORE		Marital status	
BMI		Family history	free
CO morbidity	ve	REASON OF MDT	new

(+CS)

Clinical Data

BX:

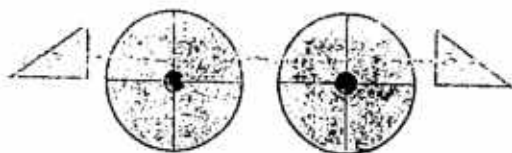
ER: 8/8 PR: 8/8 HER T: -ve KiTV: 5%

Sonomammography:

Mets Workup:

R

L



TNM

Recommendations:

45 yrs / PS = 0 / no General Crisis
MBx to bone from start since 2022
on Zoladex + Fenora

5/2025 → PD in bone

For Faslodex + Ibrance

MDT OUTCOME & PLAN

For Faslodex + Ibrance
Zoladex + Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
دار فنين فني	دار فنين فني	دار فنين فني			

دار فنين فني
00000000000000000000



جامعة المنوفية
مستشفى معهد الأورام
وحدة الطب النووي



Tc^{99m} Bone scintigraphy

الاسم	عزة مرسى ابراهيم ٣	٤٠ سنة	
التاريخ	٢٠٢٥/٥/٣١	النوع	F

TECHNIQUE:

Whole body anterior & posterior scans taken 3 hours after I.V injection of (20) mci of the tracer Tc^{99m} (MDP).

Finding:

- Increased radioactive tracer uptake over:
- D11 , D12 , L1 , L2 , L3 .
- Most (dorsal , lumber) vertebrae .
- Bilateral iliac crest .
- Both sacroiliac joint .

OPENION:

- Bone deposits for clinical and radiological correlation .
- In comparison with previous bone scan (2/2024) showed : progression disease .

Verified by:

Prof/Amira Ebrahim Aldesoky

Assistant Professor of clinical Oncology &
nuclear medicine